

Ebola Bundibugyo virus disease, Democratic Republic of the Congo, Uganda, and countries with land borders adjoining countries with documented BDBV detection

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Overall risk and confidence ¹

Overall risk				
Democratic Republic of the Congo	Uganda	Countries with land borders adjoining countries with documented BDBV detection	Rest of the African Region	Global
Very High	High	High	Low	Low

Confidence in available information				
Democratic Republic of the Congo	Uganda	Countries with land borders adjoining countries with documented BDBV detection	Rest of the African Region	Global
Moderate	Moderate	Moderate	Moderate	High

Risk statement

Following the publication of the second Rapid Risk Assessment (RRA) on 22 May 2026, the Bundibugyo virus disease (BVD) outbreak has continued to expand, particularly in the Democratic Republic of the Congo and, to a lesser extent, in Uganda.

During this period, a case was reported in a Congolese national who travelled from the Democratic Republic of the Congo, via Uganda, to the United Arab Emirates and then back to Uganda. WHO is working with public health authorities in the United Arab Emirates and Uganda to gather additional information to assess the risk of exposure and facilitate contact tracing through the National International Health Regulations (IHR) Focal Point mechanism. Following notification of the case, the United Arab Emirates authorities rapidly implemented risk assessment, contact tracing activities, follow-up of identified contacts, public health investigations, enhanced preparedness measures at points of entry, and coordination with relevant national and international partners. Epidemiological investigations to date have not identified any secondary cases, local transmission, or evidence of onward spread in the country.

Additionally, as of 6 June, the outbreak in the Democratic Republic of the Congo has expanded considerably; the number of reported affected health zones has increased from 16 to 25, while the number of laboratory-confirmed cases increased from 63 to 515 and the number of confirmed deaths from four to 91 (CFR 17.7%). The increase in the number of confirmed cases reflects both ongoing transmission and improvements in case detection through expanded testing and intensified contact tracing activities. The number of reported suspected cases decreased from 661 to 117 following the testing of a backlog of samples and subsequent reclassification of suspected cases to either confirmed cases or non-cases. So far, at least 16 healthcare workers are among the confirmed cases. Cases have been reported across all age groups, with most occurring among adults aged 20–49 years, and a slightly higher proportion among males. To date, 12 patients have recovered. The outbreak has also expanded geographically, with transmission reported in additional health zones in Ituri and North Kivu provinces. The outbreak is now reported across 25 health zones in Ituri (17), North Kivu (seven), and South Kivu (one) provinces, with new affected areas identified in both Ituri and North Kivu.

In Uganda, as of 6 June, the number of reported confirmed cases increased from two to 19 (14 imported and five acquired in Uganda), including two deaths in imported cases. All reported cases are from two districts (Kampala and

¹ Confidence refers to the level of confidence in the data/information or the quality of the evidence available at the time the RRA is conducted. Poor quality information may increase the overall perceived risk due to the incertitude in the assessment.

Wakiso). Five healthcare workers are among the confirmed cases, indicating transmission in healthcare settings. To date, all cases in Uganda have been linked to importation from the Democratic Republic of the Congo or secondary cases linked to these; there has been no documented community transmission in Uganda.

In light of the continued evolution of the outbreak and newly available information, including the increase in the number of reported cases, geographic expansion, cross-border transmission to Uganda, and ongoing response activities, this RRA has been updated. Based on these developments and the WHO [Temporary Recommendations](#) issued by the WHO Director-General following the declaration of a Public Health Event of International Concern (PHEIC) for the Ebola disease epidemic caused by Bundibugyo virus (BDBV) in the Democratic Republic of the Congo and Uganda, the risk for countries sharing land borders with countries with documented BDBV detection, currently the Democratic Republic of the Congo and Uganda, has been separated out from the risk for other countries in the African Region: the risk in countries sharing land borders remains high, while the risk for other countries in the African region is assessed as low. Countries sharing land borders with the Democratic Republic of the Congo and/or Uganda have not reported confirmed cases to date. Neighbouring countries have strengthened surveillance and point-of-entry (PoE) measures, although the extent of implementation may vary across countries. The risk globally remains unchanged and is assessed as low.

The risk in **the Democratic Republic of the Congo remains assessed as very high** due to ongoing transmission and the continued expansion of the outbreak into new health zones, increasing the potential for further national and regional spread. The key factors underpinning this assessment include:

- The outbreak has continued to expand rapidly since the previous assessment. Between **22 May** and **6 June 2026**, the number of confirmed cases increased more than eightfold from 63 to 515 cases, while the number of health zones with confirmed cases has increased by 56 % (from 16 to 25), indicating intensified transmission and geographic spread.
- The detection of cases in additional health zones in Ituri and North Kivu provinces and ongoing transmission among healthcare workers suggest that the outbreak continues to pose a very high risk of further spread within the Democratic Republic of the Congo.
- In Ituri province, 17 of the 36 health zones are now affected, with Aungba, Damas, Gety, Komanda, Lita, Mambasa and Mangala among the newly affected health zones. In North Kivu province, confirmed case detections in the Beni and Kyondo health zones have increased the number of affected health zones to seven out of 35.
- According to the most up-to-date sub-national risk stratification analysis, which will be used to further inform operational response priorities, there are a total of 159 health zones currently deemed affected or at risk; this classifies the level of community transmission and underscores the large geographic scale of response needed to control this outbreak.
 - 25 health zones with confirmed cases, including 17 'hotspot' health zones and eight 'active' health zones²
 - 19 high-risk health zones
 - 115 at-risk health zones
- Epidemiological links and the full chain of transmission are not yet clearly established, and the source of the outbreak remains under investigation.
- Retrospective investigations identified suspected viral haemorrhagic fever cases occurring back in March 2026, several weeks before outbreak confirmation, suggesting prolonged undetected transmission prior to May 2026 and the establishment of multiple disconnected transmission chains across affected communities and provinces.
- The affected area is characterized by intense population mobility linked to mining activities, trade, social ties and care seeking, with movement between rural and urban centres and across neighbouring provinces.
- Reports of patients avoiding or leaving treatment facilities, together with evidence of ongoing community mistrust of BVD prevention and response measures, raise concerns about reduced healthcare-seeking behaviour and under-detection of cases. As observed during previous Ebola disease outbreaks, community fear and misinformation have hindered case detection, contact tracing, and isolation efforts, contributing to sustained transmission. Such challenges may facilitate ongoing spread within affected communities and complicate outbreak control measures.
- Reports of numerous community deaths and challenges in the implementation and community acceptance of safe and dignified burial (SDB) practices are of concern. Traditional burial practices often involve direct contact with the deceased, which may facilitate transmission and contribute to the persistence of community-based transmission chains.
- Ongoing conflict in Ituri and North Kivu provinces restricts the movement of surveillance teams, limits the deployment of Rapid Response Teams, and hinders the secure transport of laboratory samples, as well as posing challenges to contact tracing, safe and dignified burials and control of movement of high-risk contacts in those conflict zones.

² 'Hotspot' health zones refer to those with the highest burden of active transmission among those with confirmed cases reported; 'active' refers to all other health zones with confirmed cases reported

- Limited healthcare infrastructure, combined with inadequate and insufficient Ebola Treatment Centre (ETC) and isolation capacity, may hinder effective case management and infection prevention and control measures. The mixing of suspected and confirmed cases in healthcare facilities increases the risk of nosocomial transmission and may further amplify the outbreak.
- Delays in laboratory confirmation resulting from stockouts of testing supplies and limited diagnostic capacity have hindered the timely detection, isolation, and management of cases.
- Infection among at least 16 healthcare workers, including a laboratory technician, together with low infection prevention and control (IPC) scorecard performance in affected areas, indicate a high risk of exposure in healthcare settings and significant gaps in IPC.
- Early and intensive supportive care remains the only treatment option for BVD, for which no licensed vaccine or specific therapeutics are currently available for prevention and treatment.
- Community protection capacities remain insufficient in several affected areas, including limited social listening, community feedback mechanisms, rumour management, engagement of trusted local leaders and Community Health Workers (CHWs), and systematic use of community insights to inform operational decision-making. These gaps may contribute to delayed care-seeking, underreporting, reduced acceptance of response measures and continued transmission.

The level of risk for **Uganda** is still assessed as **High** due to:

- Confirmed cross-border spread through imported cases to Uganda.
- As of 6 June 2026, Uganda had reported 19 cases linked to the outbreak in the Democratic Republic of the Congo, following the importation of two cases who travelled to Uganda to seek medical care. Among the reported cases, five are healthcare workers, indicating transmission in healthcare settings.
- Despite the suspension of passenger transport services between Uganda and the Democratic Republic of the Congo, including flights, buses, and ferries, cross-border population movement is likely to continue through informal and uncontrolled crossing points. The porous border, together with intense cross-border mobility associated with mining, trade, family visits, healthcare-seeking, displacement or population movements linked to insecurity, increases the likelihood of continued cross-border transmission.
- Potential for undetected chains of transmission in border communities.
- Preliminary analyses of population movement and cross-border mobility patterns have identified Kisoro, Kabale, Kanungu, Rukungiri, Kasese, Kikuube, Hoima, Pakwach, Nebbi, Arua, Zombo, Koboko, and Yumbe as districts at increased risk of importation and subsequent transmission of BVD from the Democratic Republic of the Congo.
- Ongoing epidemiological links along the eastern Democratic Republic of the Congo–western Uganda corridor, historically affected by Ebola outbreaks, including Bundibugyo and Sudan virus disease outbreaks.

The risk **for countries with land borders adjoining countries with documented BDBV detection**, is assessed as **high** based on the following factors:

- Sustained population mobility across porous borders linked to cross-border trade and mining activities, combined with operational constraints resulting from insecurity, displacement, and limited healthcare access, increase the risk of continued transmission and hinder outbreak control measures.
- Insufficient laboratory capacity, coupled with limited experience in BVD surveillance, case management, infection prevention and control, contact tracing, and outbreak response, may reduce the ability of some neighbouring countries to rapidly detect and contain imported cases.
- Variable levels of readiness for community engagement, community-based surveillance, social listening, rumor management and community feedback systems may limit the ability of some neighbouring countries to rapidly identify, understand and respond to community concerns following an imported case.
- There are variations in capacities and experiences across these countries.

The level of risk for the **rest of the Africa region** and at the **global level** is assessed as **low** due to:

- At present the outbreak remains geographically limited to the Democratic Republic of the Congo, with exportation of cases only to Uganda.
- No evidence suggests sustained international transmission of BVD beyond the Democratic Republic of the Congo and Uganda border areas currently.
- The exportation of cases through international travel, particularly during the asymptomatic incubation period, is possible and may be anticipated; however, this does not change the overall risk assessment, and the risk of global spread remains low.

Risk questions

Risk question		Assessment		Risk	Rationale
		Likelihood	Consequences		
Potential risk for human health?	Democratic Republic of the Congo	Highly likely	Major	Very high	The risk to human health in the Democratic Republic of the Congo is considered very high due to evidence of prolonged undetected transmission, rapid outbreak growth, ongoing transmission in community and healthcare settings, and the continued geographic expansion of the outbreak. Retrospective investigations identified suspected viral haemorrhagic fever cases dating back to March 2026, suggesting that Bundibugyo virus (BDBV) may have been circulating undetected for up to three months before the outbreak was confirmed in May. The use of diagnostic assays not validated for the detection of BDBV may have contributed to missed early cases, enabling sustained transmission in communities and healthcare facilities and allowing the outbreak to spread geographically before response measures were initiated. As a result, multiple unconnected transmission chains may have become established across affected communities and provinces.
	Uganda	Likely	Moderate	High	Since the previous assessment on 22 May 2026, the outbreak in Democratic Republic of the Congo has demonstrated rapid epidemiological growth, characterized by a more than eightfold increase in confirmed cases, nearly 23-fold increase in deaths, and continued geographical expansion, underscoring the high risk to human health. Transmission in healthcare settings remains a significant concern. Infections and deaths among healthcare workers indicate ongoing healthcare-associated transmission and gaps in IPC measures. In addition, delayed outbreak verification, movement of symptomatic individuals between health zones, and challenges in implementing dignified and safe burial practices may have facilitated transmission within both communities and healthcare settings.
	Countries with land borders adjoining countries with documented BDBV detection	Likely	Moderate	High	
	Rest of African Region	Unlikely	Minor	Low	
	Global	Unlikely	Minor	Low	Confirmed cross-border transmission and subsequent local transmission in Uganda following importation from the Democratic Republic of the Congo demonstrate the potential for elevated risk to human health.
Risk of event spreading?	Democratic Republic of the Congo	Highly likely	Major	Very high	The identification of BDBV is of particular concern because no licensed vaccine or specific therapeutics are currently available. Taken together, these factors indicate a very high risk in the Democratic Republic of the Congo and a high risk for neighbouring countries, including Uganda; risk for human health for the rest of the African Region and globally is low.
					The risk of further spread within the Democratic Republic of the Congo remains very high. Since the outbreak was first detected in May 2026, transmission has expanded substantially in both scale and geographic distribution. As of 6 June 2026, laboratory-confirmed cases have been reported across 25 health zones in three provinces

	Uganda	Likely	Moderate	High	(Ituri, North Kivu, and South Kivu). The outbreak remains concentrated in Ituri province, particularly in Bunia, Rwampara, and Mongbwalu health zones, which together account for more than 70% of confirmed cases nationally. The continued increase in cases and nearly 23-fold increase in deaths, together with the expansion into new health zones, indicates sustained transmission and a high likelihood of further spread. Several factors continue to favour further spread. The affected areas are characterised by high population mobility associated with mining activities, trade, and travel between rural communities and urban centres. Bunia functions as a major commercial, transport, and referral hub with links to other provinces and neighbouring countries. The outbreak has already spread beyond its initial focus in Ituri province into North and South Kivu, demonstrating the potential for continued geographic expansion.
	Countries with land borders adjoining countries with documented BDBV detection	Likely	Moderate	High	Cross-border transmission from the Democratic Republic of the Congo has already been documented in Uganda, including secondary transmission among contacts and healthcare workers. Frequent movement across porous borders, including through informal crossing points, increases the likelihood of additional cross-border spread despite the implementation of enhanced surveillance and screening measures at points of entry.
	Rest of African Region	Unlikely	Minor	Low	The outbreak response continues to face significant operational challenges. As of 6 June, approximately 5040 contacts had been identified for follow-up in the Democratic Republic of the Congo, however, persistent difficulties in contact tracing. Insecurity, population displacement, high mobility, and access constraints continue to hamper surveillance and response activities. Furthermore, epidemiological links between cases and complete transmission chains have not yet been fully established, and the source of the outbreak remains under investigation.
	Global	Unlikely	Minimal	Low	Taken together, the continued geographic expansion, ongoing transmission, weak contact follow-up, cross-border spread, and operational constraints indicate a very high risk of further spread within the Democratic Republic of the Congo. The risk of spread remains high in Uganda and neighbouring countries. The exportation of cases through international travel during the asymptomatic incubation period is possible and may be anticipated. However, this does not change the overall risk assessment, and the risk of global spread remains low.
Risk of insufficient control capacities with available resources	Democratic Republic of the Congo	Highly likely	Major	Very high	The risk associated with insufficient outbreak control capacity in the Democratic Republic of the Congo was previously assessed as high but is now reassessed as very high. This reassessment is driven by persistent operational and response challenges, including likely underreporting of

	Uganda	Likely	Moderate	High	cases, community mistrust of BVD response activities, low contact tracing coverage, constraints in laboratory diagnostic capacity, and limited healthcare and isolation infrastructure. These challenges are further compounded by insecurity, population displacement, difficult access to affected areas, and ongoing humanitarian pressures, which continue to hinder surveillance, case detection, and response operations.
	Countries with land borders adjoining countries with documented BDBV detection	Likely	Minor	High	Laboratory capacity remains particularly constrained, resulting in delays in case confirmation and response measures. In addition, logistical difficulties affecting surveillance, laboratory diagnostics, contact tracing, and the implementation of safe and dignified burials may further limit the effectiveness of outbreak control efforts.
	Rest of African Region	Unlikely	Moderate	Low	At the same time, the healthcare system faces multiple concurrent pressures, including recurrent outbreaks of malaria, cholera, and mpox, shortages of trained healthcare workers, limited laboratory capacity, and security incidents involving armed groups.
	Global	Unlikely	Minimal	Low	For Uganda and neighbouring countries, the risk associated with insufficient response capacity remains high. Since the detection of the outbreak, neighbouring countries have strengthened preparedness/readiness measures, including enhanced surveillance at PoE and the activation of contingency plans in line with WHO recommendations. However, continued cross-border movement through porous borders increases the likelihood of further exportation. In some countries, limited laboratory and healthcare capacity may delay the detection and containment of imported cases. The risk associated with insufficient response capacity at the regional and global levels remains low, given the low number of cases likely to be imported into any given country.

Major actions recommended by the risk assessment team

	Action
☐	Refer the event for review by IHR Emergency Committee for consideration as a PHEIC by DG (Art 12, IHR)
☐	Immediate activation of ERF response mechanism (IMS) as urgent public health response is required
☐	Recommend setting up of grading call (funding can be accessed before grading completed)
☐	Immediate support to response, but within limit of CFE (no grading recommended at this point in time)
☐	Rapidly seek further information and repeat RRA (including field risk assessment)
☒	Support Member State to undertake preparedness measures
☒	Continue to closely monitor
☐	No further risk assessment required for this event, return to routine activities

Immediate actions

- Continue the surge deployment of multidisciplinary support teams to Mongbwalu, Rwampara, Bunia and other affected, high-risk and at-risk health zones in Ituri, North Kivu and South Kivu.

- Initiate and maintain comprehensive epidemiological investigation (including support of genomic sequencing where feasible and needed) to clarify chains of transmission, identify the origin of the outbreak, and guide timely interventions.
- Implement robust contact identification, exhaustive contact listing and consistent follow-up in all affected areas.
- Strengthen surveillance and alert management including community-based surveillance and screening at all PoE, including neighbouring countries not yet affected but at high risk for importation.
- Enhance digitization of alerts, case line listing, contact listing/tracing, and laboratory data to support timely data flows for decision making by response teams.
- Strengthen Community Protection as a core response pillar, by scaling up Risk Communication and Community Engagement (RCCE), social listening, community feedback systems, infodemic management, community-based surveillance, safe and dignified burial engagement, and Community Health Worker mobilization. Establish and scale-up RCCE activities with affected and at-risk communities, including engagement of local leaders, faith-based organizations, civil society organizations, internally displaced populations, mining communities, mobile populations, and cross-border communities.
- Strengthen the collection, analysis and operational use of community insights through social listening, community feedback mechanisms and community-based surveillance, and integrate these into Emergency Operations Centre (EOC) and Incident Management System (IMS) decision-making processes.
- Reinforce IPC standards and triage in health facilities in affected and high-risk zones, including isolation capacities, safe and dignified burials, and PPE for health workers, in addition to promotion of basic IPC practices at the community level.
- Support rapid establishment and/or scaling of BVD treatment units and isolation facilities in priority locations, ensuring referral pathways and safe transport.
- Preposition and distribute essential supplies (PPE, sample collection materials, reagents, essential medicines, water, sanitation and hygiene (WASH) and safe-burial supplies) to strategic locations and healthcare facilities in affected and at-risk areas.
- Expand access to BVD diagnostic testing by decentralizing and strengthening filovirus RT-PCR capacity, ensuring adequate and sustained supply of compatible reagents and consumables, functional sample transport, and timely reporting from national and sub-national laboratories.
- Scale up readiness support in neighbouring countries, including Community Protection, RCCE, CHW preparedness, community-based surveillance, social listening, PoE measures, cross-border engagement, and readiness training packages.

Supporting information

Hazard assessment

Bundibugyo virus disease is a severe viral haemorrhagic disease caused by a virus species within the genus *Orthoebolavirus* (family *Filoviridae*). To date, six species within the *Orthoebolavirus* genus have been identified, three of which, Ebola virus, Sudan virus, and Bundibugyo virus, have been responsible for most large Ebola disease outbreaks in Africa. The current outbreak has been confirmed as caused by the Bundibugyo virus, and it is the third occurrence reported globally. The first known outbreak caused by Bundibugyo virus (BDBV) occurred in Bundibugyo District in western Uganda in 2007 and the second one was in the Democratic Republic of the Congo in 2012. During previous BVD outbreaks, the reported case fatality rates were between 30%-50%.

BVD is a zoonotic disease. Different species of bats considered the most likely natural hosts of *orthoebolaviruses*. Non-human primates such as monkeys, gorillas and chimpanzees (and more rarely other wildlife) may develop severe disease and die following infection. The exact animal reservoir for BDBV has not been conclusively identified. The virus is transmitted to humans through close contact with the infected blood or body fluids of infected wildlife and then spreads through human-to-human transmission via contact with bodily fluids or contaminated items or surfaces. Traditional burial practices involving direct contact with the body of a deceased person can also spread infection.

The incubation period varies from 2 to 21 days. Symptoms can appear suddenly and include fever, fatigue, muscle aches, headache, and sore throat. These are followed by vomiting, diarrhoea, a rash, symptoms of kidney and liver failure, and, in some cases, internal and external bleeding (e.g., bleeding gums, blood in the stool).

Treatment for BVD is mainly supportive. Early access to medical care improves the chances of survival. Currently, no treatments have been specifically approved for the treatment of BVD, although some are being advanced into clinical trials, and there is no licensed vaccine for the prevention of BVD.

Confirmation of BVD requires specialized laboratory testing, usually performed after the onset of compatible clinical symptoms. Diagnostic approaches include primarily reverse transcription polymerase chain reaction (RT-PCR) assays to detect viral RNA.

Effective outbreak control relies on the application of a set of interventions, namely clinical case management, IPC and WASH, surveillance and contact tracing, good laboratory service, safe and dignified burials, community engagement, and social mobilization. It also requires ensuring that suspected, probable, and confirmed cases and their contacts do not undertake travel, except as part of an authorized medical evacuation, alongside the implementation of exit screening, and preventing the cross-border movement of the human remains of deceased probable or confirmed BVD cases.

Exposure assessment

On 5 May 2026, WHO country office in Democratic Republic of the Congo detected social media reports of an unusual cluster of severe illness and deaths in Mongbwalu health zone (MHZ), a rural gold-mining area in Ituri province, in northeastern Democratic Republic of the Congo close to the border with South Sudan and Uganda. The affected area is characterised by high population mobility, insecurity, and intense cross-border connectivity with neighbouring countries. Clinical presentation among suspected cases included fever, headache, vomiting, profound weakness, and haemorrhagic manifestations, raising concern for a viral haemorrhagic fever. Retrospective investigation by the field team at the provincial level, covering 15 April through 13 May 2026, identified 246 suspected cases and 65 deaths (CFR 26.4%) from three health zones (Mongbwalu, Rwampara, and Bunia), including a family cluster of 15 deaths within a two-week window. After patients from MHZ sought care at the local hospital in late April (exact date not specified), eight attending healthcare workers at the hospital developed compatible symptoms between 25 April and 2 May 2026.

Twenty samples were collected from 12 patients who had travelled from MHZ to Rwampara and eight health care workers who developed symptoms after treating these patients. Following initial testing at the Bunia Public Health Laboratory using GeneXpert (not specific for BDBV), which was negative for Ebola virus, the samples were referred to the Institut National de Recherche Biomédicale (INRB) in Kinshasa for further analysis. Additional testing for other priority pathogens, including mpox, dengue, cholera, malaria, plague, COVID-19, and rotavirus, was also negative.

On 14 May 2026, WHO was notified by national authorities through the Centre d'Opération des Urgences en Santé Publique (COUSP) that 13 of the 20 samples tested were laboratory-confirmed for an Ebola disease cause by an orthoebolavirus that was not Ebola virus. Genetic sequencing results released on 15 May identified Bundibugyo virus (BDBV) as the causative agent.

On 15 May 2026, the Ministry of Health of Uganda reported one imported laboratory-confirmed case of Ebola disease caused by Bundibugyo virus in Kampala, involving a male over 50 years old from the Democratic Republic of the Congo who sought care at a local hospital on 11 May and died on 14 May 2026. The body was repatriated to Democratic Republic of the Congo on the same day, prior to laboratory confirmation. Genetic sequencing results released on 15 May established BDBV as the causative agent.

This event confirmed cross-border movement of a symptomatic case and exposure within an urban healthcare setting in Uganda: exportation of cases from Ituri province to neighboring countries. Screening, surveillance, and response measures have been activated at PoE and in high-risk districts bordering Democratic Republic of the Congo. Both Democratic Republic of the Congo and Uganda officially declared Ebola outbreaks in their respective countries on 15 May..

Genetic sequencing results released on 15 May established BDBV as the causative agent. On the same day, both Democratic Republic of the Congo and Uganda officially declared Ebola outbreaks in their respective countries.

On 18 May 2026, the US CDC confirmed that a US physician infected with Bundibugyo virus disease while providing clinical care in Ituri Province was evacuated to Germany for treatment. He tested positive on 17 May and subsequently recovered. On 6 June 2026, he was discharged from hospital. Since the last RRA, a case was reported in a Congolese national who travelled from the Democratic Republic of the Congo, via Uganda, to the United Arab Emirates and then back to Uganda. WHO is working with public health authorities in the United Arab Emirates and Uganda to gather additional information to assess the risk of exposure and facilitate contact tracing through the National IHR Focal Point mechanism. Following notification of the case, United Arab Emirates authorities rapidly implemented risk assessment, contact tracing activities, follow-up of identified contacts, public health investigations, enhanced preparedness measures at points of entry, and coordination with relevant national and international partners. Epidemiological investigations to date have not identified any secondary cases, local transmission, or evidence of onward spread in the country.

Additional Ebola alerts continue to be detected in travellers from countries with links to the Democratic Republic of the Congo and Uganda. These alerts are investigated and tested if deemed appropriate; to date no additional exported cases have been reported.

As of 6 June 2026, a cumulative of 515 confirmed cases and 91 confirmed deaths (CFR 17.7%) have been reported in the Democratic Republic of the Congo. The outbreak 25 affected health zones in three provinces: Ituri (17), North Kivu (seven), and South Kivu (one). In Ituri province (487 confirmed cases), 17 of the 36 health zones are now affected, with Aungba, Damas, Gety, Komanda, Lita, and Mangala among the newly affected health zones. In North Kivu province, the detection of confirmed cases in Beni and Kyondo health zones has increased the number of affected health zones to seven out of 35. Ongoing classification and investigation of historical cases has decreased the number

of suspected cases to 117 following reclassification. In addition, at least 16 healthcare workers are among the confirmed cases. Confirmed cases have been reported across all age groups the majority of confirmed cases occurring among adults aged 20–49 years. Men represent a slightly larger proportion of confirmed cases than women. Contact tracing has identified 5040 contacts, but due to insecurity, population movement, and access constraints in the affected areas, only 30% on average are followed up. Scale-up of contact tracing is ongoing.

According to the most up-to-date sub-national risk stratification analysis, which will be used to further inform operational response priorities, there are a total of 159 health zones currently deemed affected or at risk; this underscores the massive geographic scale of response needed to control this outbreak. Of the 25 health zones with confirmed cases, 17 are the highest-risk 'hotspot' health zones and eight are 'active' health zones. Of those without confirmed cases to date, 19 are deemed 'high-risk' health zones and an additional 115 are deemed 'at-risk' health zones.

As of 6 June 2026, Uganda has reported 19 confirmed cases of BVD including two deaths, as well as one probable case who died. All cases have been reported from two districts in the Kampala Metropolitan Area: Kampala and Wakiso. Five cases have occurred among healthcare workers, highlighting the risk of healthcare-associated transmission. As of 6 June, a total of 668 contacts had been identified and are under follow-up. To date, all cases in Uganda have been linked to importation from the Democratic Republic of the Congo or secondary cases linked to these; there has been no documented community transmission in Uganda.

Among reported cases in Uganda, adults aged 31–50 years have accounted for the majority of cases (72.7%), while three cases have been reported among persons aged 51–59 years and two among those aged 21–30 years. The predominance of cases among working-age adults is consistent with the epidemiological profile of imported cases and subsequent transmission among contacts and healthcare workers.

Context assessment

This is the 17th Ebola disease outbreak reported in the Democratic Republic of the Congo since 1976 and the second caused by BDBV. Prior to the current event, only two outbreaks caused by BDBV had been documented globally: in Uganda during 2007–2008 and in the Democratic Republic of the Congo in 2012, together resulting in more than 200 confirmed and probable cases and approximately 66 deaths. In contrast, the current outbreak has expanded rapidly since its detection in May 2026 and has already surpassed the cumulative case count of the previous BDBV outbreaks within a matter of weeks.

Retrospective investigations indicate that transmission may have been occurring several weeks before the outbreak was officially detected. Suspected viral haemorrhagic fever cases identified as early as March 2026 suggest that multiple transmission chains may have become established before response measures were initiated.

The current outbreak is occurring in eastern Democratic Republic of the Congo, a region that has repeatedly experienced Ebola outbreaks and forms part of an ecological corridor extending into western Uganda and southern South Sudan. The recurrent occurrence of Ebola outbreaks in eastern Democratic Republic of the Congo and Uganda highlights the region's vulnerability to filovirus emergence and transmission.

The outbreak continues to expand geographically, with sustained transmission concentrated in the Ituri–North Kivu corridor and increasing risks of further spread. Transmission is being facilitated by intense population mobility associated with mining activities, informal trade, displacement, and routine cross-border movement including across informal border crossings. The initially affected health zones of Mongbwalu, Rwampara, and Bunia are located in a highly connected area near the borders with Uganda and South Sudan. Bunia serves as a major transport, referral, and commercial hub linking affected provinces with neighbouring countries, increasing the likelihood of further spread if transmission is not rapidly contained.

The outbreak is occurring in a complex humanitarian and security context, in an already fragile region of the Democratic Republic of the Congo where more than 26 million people nationwide face acute food insecurity, further exacerbating the affected population's vulnerability. Malnutrition, population displacement, and the fragility of health services contribute to a high risk of infection and death. Ituri, North Kivu, and South Kivu provinces continue to experience insecurity, armed violence, and large-scale population displacement. More than 1.2 million internally displaced persons are estimated to be living in Ituri province alone; population movements associated with insecurity continue to affect surveillance, healthcare access, and response operations. The affected areas also include mining communities and informal settlements where overcrowding, limited water, sanitation and hygiene infrastructure, and restricted access to healthcare may facilitate transmission.

Community and healthcare-associated transmission remain important concerns. Reports of unexplained community deaths, delays in case detection, and challenges in implementing and gaining acceptance of safe and dignified burial

practices may have facilitated ongoing transmission. Traditional funeral and burial practices involving direct contact with the deceased remain a concern in some affected communities. At the same time, infections among healthcare workers indicate continued exposure in healthcare settings and highlight gaps in infection prevention and control (IPC) measures. Although approximately 5040 contacts have been identified in the Democratic Republic of the Congo, only around 30% were reportedly reached, indicating substantial gaps in contact tracing and increasing the likelihood of missed cases and undetected transmission chains.

Although response capacities have improved since the outbreak was detected, significant challenges remain. Gaps persist in active case search, case investigation, contact tracing, IPC readiness, community acceptance of response measures, accepted and adequate patient care. Laboratory capacity remains constrained, resulting in delays in sample testing and confirmation. Healthcare infrastructure and isolation capacity are limited, while the health system continues to face concurrent outbreaks of malaria, cholera, and mpox, shortages of trained healthcare workers, and ongoing security incidents. Funding constraints and operational challenges continue to affect the scale and effectiveness of response activities.

In Uganda, imported cases linked to the outbreak in the Democratic Republic of the Congo, together with secondary transmission among contacts and healthcare workers, demonstrate the continued risk of cross-border spread as well as lapses in IPC. The movement of confirmed cases through international travel routes further highlights the potential for wider dissemination if cases are not rapidly detected and isolated, including to vulnerable neighbouring countries such as South Sudan. In response, WHO Temporary Recommendations issued under the Public Health Emergency of International Concern (PHEIC) have been shared with States Parties through the IHR mechanism. In line with this, States Parties categorized into three levels based on risk assessment have begun to establish or strengthen coordination, surveillance, preparedness, laboratory capacity, IPC measures, and rapid response mechanisms. Enhanced exit screening and surveillance measures have also been implemented at points of entry, points of control and along major travel corridors connecting affected areas, alongside the adherence to recommendation that cases and contacts should not travel until cleared by health authorities to prevent the risk of exportation, or unless it is as part of an appropriate medical evacuation. Similarly, communication among the network of IHR National Focal Points is being facilitated via the WHO Regional Office for Africa IHR Contact Point.

Since the declaration of the PHEIC, countries sharing land borders with the Democratic Republic of the Congo and Uganda have initiated preparedness and readiness activities, including activation of coordination mechanisms, readiness assessments, surveillance strengthening, laboratory preparedness, infection prevention and control measures, Community Protection and RCCE activities, points of entry readiness, simulation exercises, and training of rapid response teams. These efforts have enhanced national preparedness and response capacities; however, significant gaps remain in several countries, particularly in subnational readiness, community engagement capacity, cross-border coordination, surge response capabilities and sustainable financing for preparedness activities.

The identification of Bundibugyo virus presents additional operational and clinical challenges in both affected countries. Currently available Ebola vaccines and therapeutics are primarily designed for Ebola virus and are not approved for use against BDBV. Furthermore, Bundibugyo virus disease remains relatively poorly characterised, with only three recognized outbreaks globally, including the current event.

Capacities and vulnerabilities related to the BVD outbreak response in the affected countries

Capacities: Democratic Republic of the Congo and Uganda	Vulnerabilities: Democratic Republic of the Congo and Uganda
Leadership and coordination - Health authorities in the Democratic Republic of the Congo and Uganda have mobilized response structures and reactivated the Ebola disease coordination committee. Surveillance - Both countries have experience with alert systems, active case search, case investigation and contact tracing from previous BVD and other filovirus outbreaks.	Coordination limitations in remote or insecure areas <u>Democratic Republic of the Congo</u> - Challenges of incentives of CHWs/volunteers - Inter-community conflict - Insufficient funding allocated - Slow feedback loops and Infodemic management in remote and insecure zones - Trust remains fragile in conflict –affected areas - Community mistrust, limited preparedness investments - Lack of capacity building, including of African Health Volunteers Corps (AVoHC) and

Laboratory - The Democratic Republic of the Congo and Uganda have previous experience in responding to Ebola disease, including possessing reference laboratories capable of diagnosing BDBV infection. <u>Democratic Republic of the Congo</u> - Capacities have been scaled up in Bunia. A decentralization plan is being implemented under INRB leadership. Two more consignments with 6 testing kits will be delivered in a week. - Validation of the kit being used (RadiOne) is ongoing but needs to be fast-tracked to understand kit performance and to support the decentralization strategy. <u>Uganda</u> - Testing capacities are available at four laboratories including two currently deployed to the bordering areas. <u>Clinical management and IPC</u> - Both countries have good clinical experience in the management of Ebola disease outbreaks with well-tested clinical systems to support effective clinical response. <u>Democratic Republic of the Congo</u> - The availability of training materials, treatment operational protocols, and job aids supports the rapid expansion of healthcare worker capacity. Furthermore, experienced clinical teams from WHO, the Ministry of Health, and partners including MSF, ALIMA, and Samaritan's Purse are already deployed to support ETCs across affected areas, strengthening clinical management and response capacity in the country. <u>Uganda</u> - The Mulago ETC is activated along with isolation capacities in other health facilities to ensure safe and timely referral of suspected cases. <u>Risk communication and community engagement - RCCE</u> - Both the Democratic Republic of the Congo and Uganda have substantial experience in Ebola-related RCCE - Both countries have experience in handling resistance during past Ebola outbreaks <u>Democratic Republic of the Congo</u> - There are functional RCCE and Infodemic coordination structures led by the MoHs at national and subnational levels	Strengthening and Utilizing Response Groups for Emergencies (SURGE) and CHWs. <u>Surveillance</u> <u>Democratic Republic of the Congo</u> - Limited human resources for contact tracing - Late reporting, delayed detection & verification - Limited epidemiological information/investigation - Need to strengthen cross border surveillance and information sharing. <u>Laboratory</u> <u>Democratic Republic of the Congo</u> - Testing capacity remains insufficient - Laboratory items such as testing kits/reagents, consumables, sample collection kits etc are currently limited. - Limited number of Bundibugyo virus-specific lab suppliers - Delays in issuing of laboratory results - Delays in swabbing and laboratory testing of deceased individuals which postpone safe and dignified burial procedure. <u>Clinical management and IPC</u> <u>Democratic Republic of the Congo</u> - Insufficient isolation and care capacity - Lack of well-formulated screening and triage procedures - Uncertain capability / capacity for management of critical illness - Lack of dedicated facilities for patient management - Unclear coordination of partners capable of delivering care - Wide geographical extent of the outbreak, which will need a rapid scale up of clinical care to support safe treatment of patients close to the community as possible - Competing medical priorities due to other routine morbidities, concurrent outbreaks, and the humanitarian situation - Threat to continuity of care secondary to HCWs infected - Clinical care infrastructural challenges in the area affected that will pose a huge threat to support safe and scalable care to affected population. - Risk of stigma from case transfers - Several patients reportedly absconded from treatment centres - Only a limited number of safe and dignified burials had been conducted - Multiple healthcare workers infections reflecting limited IPC capacity <u>Risk communication and community engagement - RCCE</u> <u>Democratic Republic of the Congo</u>
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- There is a strong operational presence of partners (WHO, IFRC, UNICEF, MSF, Alima, IFRC/Red Cross and other NGOs, etc)
- There is community engagement existing platforms (CHWs, Community relays, Red Cross Volunteers, religious groups, women and youths' associations)
- Rapid door-to-door mobilization capacity is available
- Strong community dialogue experience exists in both countries
- Existing community channels are largely trusted (SMS campaigns, Call center, WhatsApp groups, Interpersonal communication)
- RCCE materials, multilingual messages and narratives are available
- Joint messaging has been developed with partners for affected populations and specific audiences, including displaced populations, refugees, and travellers. An Ebola social media toolkit is also available and can be translated into local languages to support RCCE activities.
- Trained staff (AVoHC Surge)
- Experience in integration of RCCE linked to CBS, Surveillance, SDB, case management, etc
- Ongoing WOA, EBO-SURSY project to raise awareness of wildlife diseases in the communities, including radio campaigns focused on precaution regarding bushmeat, precaution for wild animals found sick or dead

PoE, border health and mass gatherings

- Exit screening at main airports and land borders are officially in place in Democratic Republic of the Congo and Uganda.
- [Mass gatherings guidance](#) and [PoE technical note](#) have been developed and shared with relevant stakeholders. Additional tools and documents are currently being developed.
- Risk communication materials targeting individual travellers have been developed in coordination with RCCE team
- A systematic process for monitoring travel related measures implemented by countries is being established in coordination with regional offices
- An All-Hazards Risk Assessment Tool training session had been provided for at-risk countries, including Uganda, South Sudan, Tanzania, Rwanda and Kenya.
- A major mass gathering in Uganda (Martyr's Day on 3 June) was postponed.
- Support is being provided to FIFA and PAHO in view of the upcoming FIFA World Cup that will start on 11 June 2026.
- Weekly coordination meetings are organized by WHO, with UN partners and other international organizations in the travel, transport, and tourism sectors. These are complemented by the regular sharing of disease information and technical guidance, with the aim of promoting alignment with WHO's advice on international traffic.
- PoE component has been added to readiness tools to conduct needs assessments in neighbouring countries.

- Challenges linked to complex context with community incidents
- Limited funding for RCCE interventions
- Need for scaling up resources to support decentralisation of activities
- Partners mapping and coordination to be strengthened
- Gaps in outreach and local language messaging
- Community feedback mechanism to be strengthened among partners to track community concerns, questions and damaging narratives and to integrate into the response
- Incomplete information about community perceptions
- Community rumours, misinformation, and mistrust are hindering case detection, reporting, and health-seeking behaviour
- Gaps in RCCE capacity and partner mapping may constrain the scale-up of community engagement activities
- Security challenges continue to limit access to affected communities and impede effective community engagement

PoE, border health and mass gatherings

- There is a need to scale-up PoE operations, including exit screening, and visibility on its implementation in affected areas – this is driving the use of restrictive travel measures in third countries.
- Other persistent challenges, per coordination with the International Organization for Migration (IOM) are the payment of health workers; and access to the up-to-date list of cases and contacts in a timely manner for conducting PoE exit screening.
- Despite being officially postponed by Uganda authorities, the postponement of Martyr's Day mass gathering event has not been reflected on the official website: the concern is regarding the onsite pilgrims. Despite scaled down of the event, in such settings, the high density and mobility of attendees favour close, frequent and prolonged interactions, which increase the risk of amplification of BVD transmission and potential disruption of health services.

Operational support and logistics

Democratic Republic of the Congo

- Transport and fuel shortages for field operations
- Stock-outs of essential supplies (PPE, IPC materials)

Operational support and logistics

- WHO has deployed 155 field personnel across the Democratic Republic of the Congo and Uganda to support response operations. In the Democratic Republic of the Congo, 58 personnel (19 international and 39 national staff) are deployed across key outbreak hotspots, while 97 personnel (three international staff and 94 national staff and consultants) have been deployed in Uganda through nine field teams to support surveillance, case management, laboratory, logistics, and other response activities.
- Logistics corridors from Kinshasa in the Democratic Republic of the Congo, Nairobi in Kenya, Dakar in Senegal, and other regional hubs remain operational, facilitating the movement of emergency supplies and equipment to affected areas. To date, 38 metric tonnes of medical equipment, laboratory consumables, personal protective equipment (PPE), and other emergency supplies have been transported to support response operations across Ituri, North Kivu, and South Kivu provinces. WHO has also received an additional five metric tonnes of emergency supplies through support from the United Nations Organization Stabilization Mission in the Democratic Republic of the Congo (MONUSCO).
- Rehabilitation of the Centre Médical Évangélique in Bunia has been completed, with a new 42-bed Ebola Treatment Centre expanding capacity to up to 100 beds to support the isolation and clinical management of suspected and confirmed cases.
- Cargo flights and logistics support continue to facilitate the delivery of critical response commodities despite persistent access challenges in some affected areas. Operational assessments of warehousing, stock positioning, and supply chain management continue to support commodity visibility and distribution across affected provinces.
- In Uganda, logistics support for outbreak response continues to be strengthened through support to treatment centres, isolation facilities, and points of entry. Multisectoral logistics coordination involving WHO, UNICEF, the World Food Programme, government authorities, and operational partners remains active to ensure the timely deployment of supplies and equipment.

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